

MedCare General Hospital

Department of Internal Medicine & Diagnostics

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COMPREHENSIVE MEDICAL REPORT

PATIENT INFORMATION

Patient Name:	Mr. Rajesh Kumar Sharma
Date of Birth:	14 March 1968 (Age: 57 years)
Gender:	Male
Patient ID:	MCH-2026-08741
Ward / Bed No:	General Ward B / Bed 12
Attending Physician:	Dr. Priya Mehta, MD (Internal Medicine)
Date of Admission:	20 March 2026
Date of Report:	26 March 2026

CHIEF COMPLAINTS & HISTORY

- Persistent fatigue and generalised weakness for the past 3 weeks.
- Intermittent chest pain (left-sided, non-radiating) on exertion.
- Increased frequency of urination, excessive thirst, and blurred vision.
- Productive cough with mild haemoptysis for 10 days.
- Swelling in both lower limbs progressing over 2 weeks.

Past Medical History: Known hypertensive (on amlodipine 5 mg OD) since 2015. No prior surgeries. Family history of Type-2 Diabetes (father deceased).

Allergies: Penicillin (rash), Sulphonamides.

Lifestyle: Non-smoker, occasional alcohol, sedentary occupation (accountant).

VITAL SIGNS

Parameter	Value	Normal Range	Status
Blood Pressure	158 / 96 mmHg	< 120/80 mmHg	High
Heart Rate	92 bpm	60 – 100 bpm	Normal
Respiratory Rate	20 breaths/min	12 – 20 /min	Normal
Temperature	37.8 °C	36.1 – 37.2 °C	High
Oxygen Saturation (SpO2)	94 %	95 – 100 %	Low
Body Mass Index (BMI)	29.4 kg/m ²	18.5 – 24.9 kg/m ²	High
Random Blood Glucose	218 mg/dL	< 140 mg/dL	High

DIAGNOSIS 1 — HYPERTENSION (Stage 2)

ICD-10 Code: I10 – Essential (Primary) Hypertension

Patient presents with persistent elevation of blood pressure (158/96 mmHg) despite being on antihypertensive medication (Amlodipine 5 mg OD). This classifies as Stage 2 Hypertension per the ACC/AHA 2017 guidelines. Echocardiography reveals mild concentric left ventricular hypertrophy (LVH), indicative of prolonged uncontrolled hypertension.

Relevant Investigations:

Test	Result	Reference	Status
Serum Creatinine	1.6 mg/dL	0.7 – 1.2 mg/dL	High
eGFR	52 mL/min/1.73m ²	> 60 mL/min/1.73m ²	Low
Urine Protein (24-hr)	320 mg/day	< 150 mg/day	Abnormal
ECG	LVH pattern	Normal sinus rhythm	Abnormal
Echocardiogram (EF)	52 %	55 – 70 %	Low

Management Plan: Increase Amlodipine to 10 mg OD. Add Telmisartan 40 mg OD. Low-sodium DASH diet. Monthly BP monitoring. Nephrology referral for CKD workup.

DIAGNOSIS 2 — TYPE 2 DIABETES MELLITUS

ICD-10 Code: E11 – Type 2 Diabetes Mellitus (newly diagnosed)

Patient meets WHO diagnostic criteria for Type 2 Diabetes Mellitus based on elevated random plasma glucose (218 mg/dL) and HbA1c of 8.9%. Symptoms of polyuria, polydipsia, and blurred vision are consistent with hyperglycaemia. No diabetic ketoacidosis observed. Fundoscopy reveals early background diabetic retinopathy (Grade I).

Relevant Investigations:

Test	Result	Reference	Status
HbA1c	8.9 %	< 5.7 % (Normal)	High
Fasting Blood Glucose	192 mg/dL	70 – 100 mg/dL	High
Post-Prandial Glucose	286 mg/dL	< 140 mg/dL	High
C-Peptide	2.8 ng/mL	0.5 – 2.0 ng/mL	High
Urine Microalbumin	45 mg/g Cr	< 30 mg/g Cr	Abnormal
Fundoscopy	Grade I Retinopathy	Normal	Abnormal

Management Plan: Initiate Metformin 500 mg BD (titrate to 1000 mg BD). Diabetic diet education. Regular self-monitoring of blood glucose. Ophthalmology referral. Foot care counselling. Repeat HbA1c in 3 months.

DIAGNOSIS 3 — PULMONARY TUBERCULOSIS (Active)

ICD-10 Code: A15.0 – Tuberculosis of Lung, confirmed bacteriologically

Patient presents with a 10-day history of productive cough, mild haemoptysis, and low-grade fever. Sputum AFB smear (3 samples) is positive (2+ grading). Chest X-ray demonstrates bilateral upper-lobe infiltrates with early cavitation in the right upper lobe. CBNAAT (GeneXpert MTB/RIF) confirms *Mycobacterium tuberculosis*, with no Rifampicin resistance detected (sensitive strain).

Relevant Investigations:

Test	Result	Reference	Status
Sputum AFB Smear (x3)	Positive (2+)	Negative	Abnormal
GeneXpert MTB/RIF	MTB Detected – RIF Sensitive	Not Detected	Abnormal
Chest X-Ray	Bilateral upper lobe infiltrates + cavity	Clear fields	Abnormal
ESR	88 mm/hr	< 20 mm/hr (male)	High
CRP	42 mg/L	< 10 mg/L	High
HIV ELISA	Non-Reactive	Non-Reactive	Normal
Mantoux Test	18 mm induration	< 10 mm	Abnormal

Management Plan: Initiate Category-I DOTS regimen (Isoniazid, Rifampicin, Pyrazinamide, Ethambutol – 2HRZE / 4HR). Notify to RNTCP. Respiratory isolation for initial 2 weeks. Hepatic function tests baseline and monthly. Pyridoxine 25 mg OD supplementation. Contact tracing advised.

DIAGNOSIS 4 — CHRONIC KIDNEY DISEASE (CKD Stage 3a)

ICD-10 Code: N18.3 – Chronic Kidney Disease, Stage 3

Elevated serum creatinine (1.6 mg/dL) with an eGFR of 52 mL/min/1.73m² classifies as CKD Stage 3a (mildly to moderately decreased kidney function). Likely secondary to longstanding hypertension and newly diagnosed diabetes. Bilateral pedal oedema is consistent with fluid retention due to reduced renal function. Renal ultrasound shows mildly reduced cortical echogenicity bilaterally.

Relevant Investigations:

Test	Result	Reference	Status
Serum Creatinine	1.6 mg/dL	0.7 – 1.2 mg/dL	High
Blood Urea Nitrogen (BUN)	32 mg/dL	7 – 20 mg/dL	High
eGFR (CKD-EPI)	52 mL/min/1.73m ²	> 60	Low
Serum Potassium	5.2 mEq/L	3.5 – 5.0 mEq/L	High
Serum Sodium	136 mEq/L	135 – 145 mEq/L	Normal
Serum Albumin	3.2 g/dL	3.5 – 5.0 g/dL	Low
Renal Ultrasound	Mildly echogenic cortex bilaterally	Normal echogenicity	Abnormal

Management Plan: Nephrology referral. Protein-restricted diet (0.8 g/kg/day). Avoid NSAIDs and nephrotoxic agents. Strict BP and glucose control. Monitor eGFR, electrolytes, and urine protein every 3 months. Vaccination: Pneumococcal, Hepatitis B.

DIAGNOSIS 5 — CONGESTIVE HEART FAILURE (HFrEF)

ICD-10 Code: I50.9 – Heart Failure, Unspecified

Clinical findings of bilateral pedal oedema, exertional chest discomfort, reduced SpO₂ (94%), and echocardiographic evidence of reduced ejection fraction (EF 52%) suggest early heart failure with reduced ejection fraction (HFrEF). BNP is markedly elevated at 680 pg/mL, supporting diagnosis. Exacerbated by uncontrolled hypertension and fluid retention from CKD.

Relevant Investigations:

Test	Result	Reference	Status
BNP (Brain Natriuretic Peptide)	680 pg/mL	< 100 pg/mL	High
Echocardiogram EF	52 %	55 – 70 %	Low
Chest X-Ray (CXR)	Cardiomegaly, mild pulmonary oedema	Normal	Abnormal
Troponin-I	0.04 ng/mL	< 0.04 ng/mL	High
ECG	LVH + non-specific ST changes	Normal sinus rhythm	Abnormal
6-Minute Walk Test	310 meters	> 400 meters	Low

Management Plan: Add Furosemide 40 mg OD (loop diuretic). Initiate Carvedilol 3.125 mg BD (titrate slowly). Fluid restriction 1.5 L/day. Daily weight monitoring. Cardiology referral. Sodium-restricted diet. Repeat echocardiogram in 6 weeks.

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SUMMARY OF DIAGNOSES & MEDICATIONS

#	Diagnosis	ICD-10	Severity
1	Hypertension – Stage 2	I10	High
2	Type 2 Diabetes Mellitus (New Onset)	E11	High
3	Pulmonary Tuberculosis (Active, Drug-Sensitive)	A15.0	Abnormal
4	Chronic Kidney Disease – Stage 3a	N18.3	Abnormal
5	Congestive Heart Failure (HFrEF)	I50.9	High

PRESCRIBED MEDICATIONS

#	Drug	Dose	Route	Frequency	Duration
1	Amlodipine	10 mg	Oral	Once daily (morning)	Ongoing
2	Telmisartan	40 mg	Oral	Once daily (morning)	Ongoing
3	Metformin	500 mg → 1000 mg	Oral	Twice daily (with meals)	Ongoing
4	Isoniazid (H)	300 mg	Oral	Once daily (empty stomach)	6 months
5	Rifampicin (R)	600 mg	Oral	Once daily (empty stomach)	6 months
6	Pyrazinamide (Z)	1500 mg	Oral	Once daily	2 months
7	Ethambutol (E)	1200 mg	Oral	Once daily	2 months
8	Pyridoxine (Vit B6)	25 mg	Oral	Once daily	6 months
9	Furosemide	40 mg	Oral	Once daily (morning)	Review in 4 wks
10	Carvedilol	3.125 mg	Oral	Twice daily	Ongoing (titrate)

FOLLOW-UP & REFERRALS

Cardiology:	Within 2 weeks for HFrEF evaluation and medication titration.
Nephrology:	Within 2 weeks for CKD staging and renal protection strategy.
Ophthalmology:	Within 1 month for diabetic retinopathy grading.
RNTCP / DOTS Centre:	Immediate referral for supervised TB treatment.
Dietician:	Within 1 week for integrated diet plan (Diabetic + Cardiac + Renal diet).
Next OPD Review:	5 April 2026 with Dr. Priya Mehta.

PHYSICIAN'S NOTES & DISCLAIMER

This report is based on clinical assessment, diagnostic investigations, and evidence-based guidelines available at the time of evaluation. All diagnoses are provisional pending specialist review. This document is strictly confidential and intended solely for the patient and treating medical team. Any reproduction or use outside the treating context requires written

consent.

Dr. Priya Mehta,
MD Internal Medicine Reg. No: MCI-2009-41872 Date: 26
March 2026

Countersigned
by: Dr. Anil Verma, MD (HOD) Internal Medicine Dept.
Date: 26 March 2026